

CLINICAL FINDINGS

Lichen sclerosus (LSc) is discussed in detail in Chap. 63. The symptoms and signs of male genital LSc are summarized in Table 75-3 and partially illustrated in Figs. 75-15 and 75-16. The classic genital morphologic presentation of LSc is atrophic white patches or plaques. However, lilac, slightly scaly patches with telangiectasia and sparse purpura—similar to the classic clinical manifestations of extragenital LSc—may also be observed. Urethral involvement may be more common and extensive than previously recognized.

Most cases of LSc can be diagnosed clinically. If there is diagnostic uncertainty, a biopsy should be performed. The histologic findings are discussed in Chap. 63.

DIFFERENTIAL DIAGNOSIS

Disorders from which LSc must be differentiated are listed in Box 75-6.

COMPLICATIONS

Squamous cell carcinoma (SCC) of the penis is the most concerning complication of LSc. A transformation rate between 4% and 9.5% has been reported, with a latent period of 10 to 30 years. Glans involvement with LSc is a significant risk factor. Carcinoma complicating LSc accounts for approximately 25% to 50% of all penile cancer cases. A pseudohyperplastic preputial SCC associated with LSc has been described; this variant is an extremely well-differentiated, non-verruciform, often multicentric form of SCC.

Treatment of LSc is discussed in Chap. 63 and in online resources.

Non-Specific Balanoposthitis

Non-specific balanoposthitis is considered either a diagnosis of exclusion or a descriptive term reflecting limitations in diagnostic precision. Sexually transmitted infections, immunosuppression, diabetes, and primary dermatoses

such as psoriasis, seborrheic dermatitis, Zoon balanitis (ZB), lichen sclerosus (LSc), lichen planus (LP), genital warts, or carcinoma in situ must be ruled out. A biopsy is essential for accurate diagnosis.

In approximately 10% of biopsy specimens from patients presenting with balanoposthitis, non-specific histopathologic features are found. While genitourinary physicians often diagnose candidiasis, dermatologists argue that *Candida* infection typically complicates existing inflammatory mucocutaneous conditions and represents a secondary, opportunistic phenomenon rather than a primary cause.

Preputial dysfunction is likely both a cause and consequence in most cases of non-specific balanoposthitis, and many patients may have undiagnosed LSc as the underlying condition. Treatment can be challenging, and the condition may not respond to local hygiene measures, soap substitution, topical steroids, or topical and systemic antibiotics.

The definitive treatment is circumcision, which is curative in most cases.